

COVID-19 Vaccination, 2026/27 Season

Patient Group Direction, version 007, issued 11 September 2026. Comirnaty XFG, Comirnaty LP.8.1 (existing stock only), Spikevax LP.8.1 and Nuvaxovid JN.1. Privately funded vaccination, aged 12 years and over.

Which product to use

- **Comirnaty XFG is the vaccine of choice under this PGD. It is the 2026/27 seasonal formulation approved for use in the United Kingdom.**
- **Comirnaty LP.8.1 remains authorised under this PGD, but only from stock already held. Pfizer ceased supply of LP.8.1 on 9 September 2026, so no further stock can be obtained. This arm ends when existing stock is exhausted or reaches its labelled expiry date, whichever comes first.**
- Both are current UK-licensed products. LP.8.1 is not out of date or substandard: the World Health Organization named monovalent LP.8.1 as its preferred antigen for the 2026/27 season, with XFG as an acceptable alternative, while the MHRA and EMA moved to XFG. Using remaining LP.8.1 stock is clinically appropriate.
- **Where both are held, give Comirnaty XFG to anyone who is immunosuppressed or aged 75 years and over. Comirnaty LP.8.1 may be used for everyone else until that stock is gone.**
- **Where Comirnaty LP.8.1 is given, tell the individual that they are receiving the previous seasonal formulation and that the current one is XFG, and record that this was explained and accepted.**
- **Check the variant designation printed on the syringe label before every administration. XFG and LP.8.1 are both 30 micrograms in 0.3 mL and the packaging is closely similar. Do not identify the product by carton appearance alone.**
- Spikevax LP.8.1 is for use where Comirnaty is unavailable. Nuvaxovid JN.1 is for use where the mRNA vaccines are unavailable or unsuitable. The dose volume differs between these products and Comirnaty. See Appendix 1.
- Do not mix presentations within a single administration, and do not transfer vaccine between syringes.

Guideline summary, COVID-19 vaccination 2026/27

- The cohorts eligible for NHS vaccination in England in autumn 2026 are adults aged 75 years and over, residents in care homes for older adults, and individuals aged 6 months and over who are immunosuppressed as defined in the COVID-19 chapter of the Green Book. Eligibility is unchanged from the spring 2025, autumn 2025 and spring 2026 campaigns.
- **This PGD covers privately funded vaccination, whether or not the individual is eligible under the NHS programme. It does not replace the national programme. Individuals who are eligible must be told they can be vaccinated free of charge on the NHS before any private supply proceeds.**
- Vaccine strains are updated periodically to match circulating variants. For the 2026/27 season, regulators recommended a monovalent XFG formulation, XFG being a JN.1-lineage Omicron subvariant. Confirm the formulation supplied against the pack and the current SPC before administration.
- A minimum interval of 3 months is required between COVID-19 vaccine doses.
- Following confirmed COVID-19 infection, defer until the individual has recovered. A 4 week interval from a positive test or symptom onset is commonly applied, and 12 weeks in children and young people aged 5 to 17 who are not in a risk group.
- COVID-19 vaccine may be co-administered with influenza vaccine at a separate site.
- **This PGD must be reviewed immediately if the approved seasonal formulation changes again, without waiting for the expiry date below.**

COVID-19 vaccination

Patient Group Direction for the administration of Comirnaty XFG, Comirnaty LP.8.1, Spikevax LP.8.1 or Nuvaxovid JN.1 for active immunisation against COVID-19 caused by SARS-CoV-2, in individuals aged 12 years and over.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- All users must be familiar with the current Summary of Product Characteristics for the specific product and variant presentation they are administering, and with the COVID-19 chapter of the Green Book (chapter 14a).
- All users must have completed immunisation training in line with the National Minimum Standards and Core Curriculum for Immunisation Training, updated for the current season.
- All users must be competent in the recognition and immediate management of anaphylaxis, and hold current basic life support training.
- All users must be competent in vaccine handling and cold chain management, including the action to take on a cold chain excursion.
- All users must be able to read and act on the variant designation printed on the syringe label and carton. Comirnaty XFG and Comirnaty LP.8.1 are both 30 micrograms in 0.3 mL and their packaging is closely similar. The variant is the only thing that distinguishes them.
- All users must previously have supplied or administered medicines under a PGD.
- All users must understand capacity and consent and be aware of the Mental Capacity Act 2005.
- All users must be up to date with CPD and appraisal, and with MHRA safety alerts relating to these products.
- All users must hold appropriate indemnity covering this service, and must work in compliance with the SOPs of their own employer.

The PGD

Indication	Active immunisation of individuals aged 12 years and over for the prevention of COVID-19 caused by SARS-CoV-2, using one of the products listed in Appendix 1, in accordance with the current Summary of Product Characteristics for that product and variant, and the COVID-19 chapter of Immunisation Against Infectious Disease (the Green Book, chapter 14a). Comirnaty is the vaccine of choice, with Spikevax and Nuvaxovid available where Comirnaty stock is unavailable. This PGD covers privately funded vaccination, whether or not the individual is eligible under the NHS programme.
Inclusion criteria	• Aged 12 years and over. There is no upper age limit. • Requires COVID-19 vaccination and either does not qualify for NHS vaccination, or qualifies but prefers to be vaccinated privately having been told of their NHS entitlement. • At least 3 months since the last COVID-19 vaccine dose. • Valid informed consent obtained, from a person with parental responsibility where the individual is under 16 and not assessed as Gillick competent, with the basis of any Gillick assessment recorded. • A product listed in Appendix 1 is held, in date, and licensed for the age of the individual. • No exclusion criterion present.
Exclusion criteria	Refer, do not vaccinate, where any of the following applies. • Aged under 12 years. Refer to the GP or a service commissioned to vaccinate children. Age-specific presentations and dose volumes are required and are outside this PGD, even where the vial held is licensed from 6 months. • Known hypersensitivity to the active substance or to any excipient listed in the current SPC, including polyethylene glycol (PEG) for the mRNA vaccines and polysorbate 80 for Nuvaxovid. • History of anaphylaxis to a previous dose of the same vaccine or to any of its components.

	• History of myocarditis or pericarditis following a previous dose of any COVID-19 vaccine. Refer for specialist advice. No further dose of any product, mRNA or Nuvaxovid, is given under this PGD. • Acute severe febrile illness. Postpone until recovered. A minor infection without fever is not a contraindication. • Confirmed current COVID-19 infection. Defer until recovered. • Less than 3 months since the last COVID-19 vaccine dose, unless a shorter interval is specifically advised in national guidance for that individual. • Bleeding disorder where intramuscular injection has not been assessed as safe by a clinician familiar with the individual's bleeding risk. • A primary course in an individual who is unvaccinated and immunosuppressed, or any schedule requiring more than one dose in the season. Refer. • Valid informed consent not obtained, or consent from a person with parental responsibility not available where required. • The only stock held has passed its labelled expiry date, or is Comirnaty LP.8.1 and the individual is immunosuppressed or aged 75 years and over and Comirnaty XFG is not held. Rebook rather than substitute.
Cautions	Anaphylaxis. Facilities and trained staff for the management of anaphylaxis must be available, with immediate access to adrenaline (epinephrine) 1 in 1,000 injection and a telephone. Observe every individual for 15 minutes after vaccination, seated, and record that the observation period was completed. Ensure procedures are in place to avoid injury from faints. Anxiety related reactions, including vasovagal reactions (syncope), hyperventilation or stress related reactions such as dizziness, palpitations, increased heart rate, altered blood pressure, paraesthesia, hypoaesthesia and sweating, may occur in association with the vaccination process itself. These are temporary and resolve on their own. Syncope can occur, particularly in adolescents. Advise the individual to bring any symptoms to the attention of the vaccinator. Myocarditis and pericarditis have been reported very rarely after mRNA vaccines, most often in younger males after a second dose. Advise the individual to seek urgent medical attention if they develop chest pain, shortness of breath, palpitations or a fluttering heartbeat after vaccination. Capillary leak syndrome. A few flare-ups have been reported in the first days after vaccination with Spikevax. Be alert to hypotension and oedema. In an individual with a history of capillary leak syndrome, vaccination should be planned in collaboration with appropriate medical experts. Pregnancy and breastfeeding. Pregnancy is not itself an eligible group in the 2026/27 NHS programme. The Green Book supports COVID-19 vaccination in pregnancy where the individual is otherwise eligible, and vaccination is safe while breastfeeding. Where the individual is pregnant, confirm the vaccine and indication against current national guidance before proceeding. Immunosuppression. The immune response may be reduced. Vaccination is still recommended and the individual should be advised that protection may be limited. These individuals are eligible under the NHS programme and must be told so. Give Comirnaty XFG in preference where it is held. Anticoagulation and bleeding disorders. Individuals on stable anticoagulation may be vaccinated intramuscularly with a 23 gauge or finer needle, with firm pressure applied without rubbing for at least 2 minutes. Advise on the risk of haematoma.
Actions if the individual is excluded or declines	Advise on alternative options and how these can be accessed, including their GP practice and, where eligible, the NHS programme. Explain the benefit of vaccination and the risks of COVID-19. Document the advice given and the

	decision reached. Inform or refer to the GP as appropriate. Where myocarditis or pericarditis followed a previous dose, refer for specialist advice.
Dose and frequency	A single dose per campaign, at the volume stated in Appendix 1 for the product held. Minimum interval of 3 months from the previous COVID-19 vaccine dose. This PGD does not cover primary courses in individuals who are unvaccinated and immunosuppressed, or any schedule requiring more than one dose in the season.
Quantity to be administered	One dose of the selected product, at the volume stated in Appendix 1. This PGD does not permit supply of vaccine to the individual for administration elsewhere.
Maximum or minimum treatment period	One dose for the 2026/27 season. This PGD expires on the date shown in the authorisation block and must be reviewed and reissued, including confirmation of the approved seasonal formulation. It must also be reviewed immediately if the approved formulation changes before that date.
Route and method of administration	• Administer by intramuscular injection into the deltoid muscle of the upper arm. • Check the variant designation on the syringe label immediately before administration and again against the record made. • Prepare in accordance with the SPC for the presentation held. Comirnaty XFG and Comirnaty LP.8.1 pre-filled syringes must not be diluted. • Inspect visually and do not administer if the appearance differs from that described in the SPC. • Where given with other vaccines, including influenza vaccine, use separate sites, preferably different limbs, or at least 2.5 cm apart in the same limb. Record the site of each.
Yellow Card reporting	Report all suspected adverse reactions via https://yellowcard.mhra.gov.uk . State the variant designation of the product given, not just the brand. Inform the GP.
Records to be kept	• That valid informed consent was given, and from whom. Where the individual is under 16, record the name and relationship of the person with parental responsibility, or the basis of the Gillick competence assessment. • Individual name, address, date of birth and registered GP. • Brand name AND variant designation of the vaccine given, written in full, for example "Comirnaty XFG" or "Comirnaty LP.8.1". Recording only "Comirnaty" is not sufficient. • Batch number and expiry date. A recall cannot be actioned without the batch number. • Date of administration, dose, route and anatomical site used. • Where Comirnaty LP.8.1 was given, that the individual was told this is the previous seasonal formulation, that Comirnaty XFG is the current one, and that they consented on that basis. • The date of the previous COVID-19 vaccine dose, where known. • Name and registration number of the healthcare professional administering. • Advice given, including advice given where the individual is excluded or declines. • Details of any adverse reaction and the action taken. • That the vaccine was administered under this PGD. Records signed, dated, legible and contemporaneous. Electronic records are acceptable. Inform the individual's GP. Adults aged 18 and over: keep for 8 years. Under 18s: keep until the 25th birthday, or the 26th birthday where the individual was 17 when treatment finished.
Storage and cold chain	Store in accordance with the current SPC for the product and presentation held, including any thawing and in-use stability requirements for the mRNA

	vaccines. Do not freeze Nuvaxovid. Store Comirnaty XFG and Comirnaty LP.8.1 in physically separate, clearly labelled locations within the fridge for as long as both are held. Do not store them mixed. The two look alike and are distinguished only by the variant designation on the label. COLD CHAIN EXCURSION: vaccine exposed to conditions outside the stated range must be quarantined and risk assessed in accordance with UKHSA Vaccine Incident Guidance before any further use. Do not administer excursion stock until that assessment is complete. Check remaining Comirnaty LP.8.1 stock against its labelled expiry date weekly. It cannot be replaced once used or expired.
Disposal	Dispose of used syringes, needles, vials and any discharged vaccine in a UN-approved puncture-resistant sharps container in accordance with local arrangements and HTM 07-01.
Co-administration	May be given at the same time as influenza vaccine and other vaccines at a separate site. Concomitant administration of Nuvaxovid JN.1 with other vaccines has not been studied; where it is used, consider giving it separately. The immune response may be reduced in individuals receiving immunosuppressive treatment, which is not a reason to withhold vaccination.

The medicine

Legal category	POM.
Black triangle	All products in this PGD carry the black triangle and are subject to additional monitoring. Report all suspected adverse reactions via the Yellow Card scheme.
Off-label use	This PGD does not authorise off-label use. Administration must be within the licensed age, presentation and dose stated in the current SPC for the product and variant held.
Drug interactions	Refer to the current SPC for each product. May be given at the same time as influenza vaccine and other vaccines at a separate site. The immune response may be reduced in individuals receiving immunosuppressive treatment.
Adverse effects	Very common and common: injection site pain, swelling and redness, fatigue, headache, myalgia, arthralgia, chills, fever, nausea and lymphadenopathy. Rare: hypersensitivity reactions including anaphylaxis. Very rare: myocarditis and pericarditis, reported mainly after mRNA vaccines. Capillary leak syndrome flare-ups have been reported after Spikevax.

Information for the patient

Written information	Offer the marketing authorisation holder's patient information leaflet for the product and variant administered. The leaflet for Comirnaty XFG is not the leaflet for Comirnaty LP.8.1; give the correct one. Provide a written record of the vaccine given with the date, brand, variant designation and batch number.
Counselling	• Where Comirnaty LP.8.1 is given: explain that this is the previous seasonal formulation, that the current formulation is XFG, that both are licensed in the UK and that the World Health Organization named LP.8.1 as a preferred antigen for this season. Record that this was explained and accepted. • Seek urgent medical attention for chest pain, shortness of breath, palpitations or a fluttering heartbeat after vaccination. • Injection site soreness, tiredness, headache, aching muscles, chills and mild fever are common in the first day or two and settle on their own. • Protection develops over about 1 to 2 weeks and wanes over time. Vaccines do not provide 100% protection, and vaccination does not

	remove the need to seek advice if you become unwell. ● Report any suspected side effect yourself via the Yellow Card scheme at https://yellowcard.mhra.gov.uk. ● Where you are eligible for NHS vaccination, you can have this vaccine free of charge on the NHS.
Follow-up	Advise the individual to seek medical advice in the event of an adverse reaction and to report it via the Yellow Card scheme. Seek urgent medical attention for the cardiac symptoms listed above. For a routine query about the vaccine, contact the pharmacy.

Appendix 1: Vaccines covered by this PGD

Confirm the presentation, licensed age range and dose volume against the current SPC before administration. The dose volume is not the same for all four products.

Comirnaty XFG 30 micrograms/dose dispersion for injection in pre-filled syringe (BioNTech/Pfizer)	• Licensed 12 years and over. • 30 micrograms in 0.3 mL, intramuscular, single dose. Do not dilute. • VACCINE OF CHOICE. The 2026/27 seasonal formulation. • Give in preference to LP.8.1 for anyone immunosuppressed or aged 75 years and over. • Paediatric presentations exist for 6 months to 11 years and are outside this PGD.
Comirnaty LP.8.1 30 micrograms/dose dispersion for injection in pre-filled syringe (BioNTech/Pfizer)	• Licensed 12 years and over. • 30 micrograms in 0.3 mL, intramuscular, single dose. Do not dilute. • EXISTING STOCK ONLY. Pfizer ceased supply on 9 September 2026. Use until stock is exhausted or reaches its labelled expiry date, whichever comes first. Do not obtain further stock to extend this arm. • Do not give to anyone immunosuppressed or aged 75 years and over where Comirnaty XFG is held. • Tell the individual this is the previous seasonal formulation and record that you did.
Spikevax LP.8.1 0.1 mg/mL dispersion for injection (Moderna)	• Licensed 6 months and over. Restricted to 12 years and over under this PGD. • At 12 years and over: 50 micrograms in 0.5 mL, intramuscular, single dose. • Multidose vial containing 5 doses of 0.5 mL or 10 doses of 0.25 mL. The 0.25 mL dose is a paediatric dose and must not be given under this PGD. • Use where Comirnaty is unavailable.
Nuvaxovid JN.1 dispersion for injection in pre-filled syringe (Sanofi)	• Licensed 12 years and over. • 0.5 mL, intramuscular, single dose. • Recombinant protein vaccine with Matrix-M adjuvant. Contains polysorbate 80. • Use where the mRNA vaccines are unavailable or unsuitable. • Do not freeze.

Product names, licensed ages and dose volumes above were checked against the UK Summary of Product Characteristics for each product on 8 September 2026. Check the current SPC before relying on them.

Appendix 2: Key references

- Immunisation Against Infectious Disease (the Green Book), COVID-19 chapter (chapter 14a).
- JCVI statement on COVID-19 vaccination in autumn 2026 and spring 2027, 16 July 2025, and the government response accepting that advice.
- Regulatory recommendations on the antigenic composition of COVID-19 vaccines for the 2026 to 2027 season, including the WHO TAG-CO-VAC statement of 16 May 2026.
- Summary of Product Characteristics for Comirnaty XFG, Comirnaty LP.8.1, Spikevax LP.8.1 and Nuvaxovid JN.1, current versions.
- UKHSA Vaccine Incident Guidance.
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions.
- British National Formulary.

Authorisation

Version	v007
Supersedes	Version 006, 11 September 2026
Valid from date	11 September 2026
Expiry date	31 July 2027

Authorised on behalf of Get Real Health by the Medical Director and the Head Pharmacist. Their signatures for this version are on the signed authorisation page at the end of this document.	

Adoption by the employing organisation

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: Job title and organisation: Signature: Date:
Professional group signatory	Name: Job title and organisation: Signature: Date:

Change history

Version	v005
Date	8 September 2026
Changes	• The practitioner Agreement to practise page, the premises block and the practitioner signature table are restored. Every version 001 document carried them; no document produced by the current generator did, through one omission in one function, which left 15 live documents with no page for an individual practitioner to sign. Raised as blocking by an adopting pharmacy: NICE MPG2 expects a record of the individuals authorised to work under a PGD. The standard governance requirements are restored at the same time: SPC and BNF familiarity, MHRA safety alerts, CPD and appraisal, indemnity, and capacity and consent, all of which were in version 001 and were lost in the rewrite. This version changes no clinical content. • Comirnaty XFG added as the vaccine of choice, being the 2026/27 seasonal formulation approved for the United Kingdom. • Comirnaty LP.8.1 added as a time-limited arm for existing stock only, ending when stock is exhausted or expires. Pfizer ceased supply on 9 September 2026. • Comirnaty JN.1 removed. Version 003 named Comirnaty JN.1 in the vaccines table while its own summary page stated the 2026/27 formulation was XFG. No pharmacy held JN.1 stock. • Spikevax corrected from the JN.1 formulation to Spikevax LP.8.1, the current UK presentation, with the paediatric 0.25 mL dose expressly excluded. • Nuvaxovid marketing authorisation holder corrected from Novavax to Sanofi. • Requirement added to record the variant designation, not just the brand name, in the patient record, on the Yellow Card and on the written record given to the individual. • Requirement added to tell the individual, and to record, where Comirnaty LP.8.1 is given in place of XFG. • Requirement added to store Comirnaty XFG and Comirnaty LP.8.1 separately and clearly labelled while both are held, and to check the variant on the syringe label before every administration. • Preference rule added: Comirnaty XFG for anyone immunosuppressed or aged 75 years and over where both formulations are held. • Review trigger added requiring immediate review if the approved seasonal formulation changes before the expiry date. • Vaccines moved to Appendix 1 with the four products set out separately, and a references appendix added. • Expiry aligned to the end of the 2026/27 season, including the spring 2027 campaign, in place of 31 January 2027.

Previous versions

001, 25 August 2025	Development and issue of new PGD. Nuvaxovid and Comirnaty, 12 years and over.
002, 30 July 2026	Reissued for the 2026/27 season. Spikevax added alongside Comirnaty and Nuvaxovid, with Comirnaty named as the vaccine of choice. KP.2 and LP.8.1 variant wording replaced. Eligibility, minimum 3 month interval, myocarditis and pericarditis exclusion and counselling, pregnancy, immunosuppression and NHS entitlement wording added. Age range unchanged at 12 years and over.
003, 25 August 2026	Issued as version 003 with a valid from date of 1 September 2026. No change history entry was recorded for this version in the document itself.

Standard requirements for every practitioner working under this PGD

All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.

All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.

All users must be up to date with their CPD requirements and appraisal.

All users must hold appropriate professional indemnity covering this service.

All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them.

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Premises name
 Address
 Postcode:
 ODS code, if applicable

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

[illegible]

Version and change record

Version: v007

Issued: 11 September 2026

Supersedes: Version 006, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Myocarditis or pericarditis after any previous COVID-19 vaccine dose now excludes every product under this PGD, matching the exclusion header (refer, do not vaccinate); the bullet previously said only that a further mRNA dose was not given.

Observation after vaccination is now 15 minutes seated for every individual, as in the other vaccine PGDs, rather than only where there is a history of allergy.

Pregnancy caution reworded to state that pregnancy is not itself an eligible group in the 2026/27 NHS programme.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v006

Issued: 11 September 2026

Supersedes: Version 005, 9 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Signed on behalf of Get Real Health

Version v007, 11 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		11 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		11 September 2026

Both authorising signatories reviewed this version together on 11 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.